Document Identifier: **PMC8712907**

SciScore Report

Below you will find your SciScore report containing three tables. Your score is calculated based on adherence to scientific rigor criteria (Table 1) and identification of key biological resources (Table 2). Table 3 contains statistical tests and oligonucleotides but is not scored. If SciScore makes any mistakes, please contact us to help us learn and improve.

Table 1: Rigor Adherence Table

Ethics
Consent: Each patient provided written informed consent prior to study inclusion.
IRB: The local ethics committee of the University of Erlangen-Nuremberg approved the study protocol.
Inclusion and Exclusion Criteria
Study population Individuals, aged between 18 and 85 years, were included in the study, if they had a HF with reduced (HFrEF) or mid-range ejection fraction (HFmrEF), according to the ESC guidelines definition, in stable condition New York Heart Association (NYHA) II-III. 12 Patients could have diabetes or not however, key exclusion criteria were any other form of diabetes than type 2 diabetes, the use of insulin, or any SGLT2 inhibitor 10 weeks prior to inclusion.
Attrition
not detected.
Sex as a biological variable
not detected.
Subject Demographics
Age: Study population Individuals, aged between 18 and 85 years, were included in the study, if they had a HF with reduced (HFrEF) or mid-range ejection fraction (HFmrEF), according to the ESC guidelines definition, in stable condition New York Heart Association (NYHA) II-III. 12 Patients could have diabetes or not however, key exclusion criteria were any other form of diabetes than type 2 diabetes, the use of insulin, or any SGLT2 inhibitor 10 weeks prior to inclusion.
Randomization
Methods Study design This was an investigator initiated, double-blind, randomized, placebo-controlled, parallel-group, prospective phase II single centre study, performed at the Clinical Research Unit (CRC) of the Department of Nephrology and Hypertension, University Hospital Erlangen-Nuremberg, Germany, in order to analyse changes of vascular function and of tissue sodium content in patients with HF after SGLT2 inhibition with empagliflozin.
Blinding
not detected.

Power Analysis	
not detected.	
Replication	
not required.	
Cell Line Authentication	
Authentication: Over the past years, central BP devices emerged, and in 2017, the Artery Society released recommendations on validation protocols for these devices and demanded invasive central BP as being the reference. 16 One of these validated, highly reliable systems is the SphygmoCor XCEL System (AtCor Medical, Sydney, Australia), which was used in this study to assess vascular parameters under resting conditions.	
Protocol Information	
Identifiers: The study was registered at http:www.clinicaltrials.gov (NCT03128528).	NCT03128528

Table 2: Key Resources Table

Your Sentences	REAGENT or RESOURCE	SOURCE	IDENTIFIER
Software and Algorithms			
Statistical analysis was performed using SPSS Statistics software, version 24.0 (IBM Corporation, Chicago, IL, USA).	SPSS		Suggestion: SPSS, RRID:SCR_002865(link)

Other Entities Detected

Your Sentences	Recognized Entity
Statistical Tests	
Statistical significance of changes between baseline and 1 or 3 months treatment with empagliflozin or placebo were determined using paired t-test, whereas for differences between 1 or 3 months treatment with empagliflozin compared with placebo, an unpaired t-test was used.	t-test

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For a full description of scored criteria and tips for improving your score, please see https://www.scicrunch.com/sciscorereport-faq